

Section 6

CFSR Longitudinal Record — Rounds 1–4

Version 2.0 — Preliminary Draft

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CFSR LONGITUDINAL FORENSIC ANALYSIS REPORT

(Companion Document to the Petition to Audit — Iowa DHHS Child Welfare System)

Version 1.0 — May 30, 2026

[Round 1 fully verified: Statewide Assessment (pp.1–155), Federal Final Report (Oct 14, 2003), PIP (pp.1–145, Aug 4, 2004)]

[Round 2 fully verified: Statewide Assessment (pp.1–198, March 2010), Federal Final Report (Feb 2011), PIP (Oct 1, 2011)]

[Round 3 fully verified: Statewide Assessment (pp.1–149, Feb 15, 2018), Federal Final Report (2018), PIP (May 13/14, 2020)]

6.1 PURPOSE AND SCOPE

This section presents the documented federal findings from Iowa's first three Child and Family Services Review (CFSR) cycles, spanning 2003 through approximately 2021. Content is drawn directly from the Statewide Assessment, Federal Review Final Report, and Performance Improvement Plan (PIP) produced for each round.

This section functions as a factual evidentiary foundation. Analytical conclusions derived from cross-document comparison and longitudinal pattern analysis are reserved for Section 1 (Executive Synthesis) and the Systemic Failure Matrix (Attachment A31). Inferential language has been excluded from this section in accordance with GAGAS evidentiary standards.

Source attachments: A2 (Round 1), A3 (Round 2), A4 (Round 3).

6.2 CFSR ROUND 1 (2003–2004)

6.2.1 Outcome Determinations

The federal onsite review was conducted the week of May 19, 2003 and the Final Report was issued October 14, 2003. Findings were derived from reviews of 50 cases across three sites

(Linn County/Cedar Rapids, Polk County/Des Moines, and Woodbury County/Sioux City), supplemented by stakeholder interviews and the State Data Profile (Final Report, p.15).

Iowa was determined to be in substantial conformity with 2 of 7 primary outcomes:

Safety Outcome 2 — Children are safely maintained in their homes: substantially achieved in

93.5% of cases (Final Report, p.3 and p.21).

Well-Being Outcome 2 — Children receive appropriate services for educational needs: substantially achieved in 92.7% of applicable cases (Final Report, p.6 and p.48).

Iowa was determined NOT to be in substantial conformity with the following 5 outcomes:

Safety Outcome 1 — 82.9% substantially achieved (below the 90% threshold) (Final Report, p.3 and p.17).

Permanency Outcome 1 — 50.0% substantially achieved. Five of six indicators rated Areas Needing Improvement (Final Report, p.4 and p.24).

Permanency Outcome 2 — 82.1% substantially achieved (Final Report, p.5 and p.33).

Well-Being Outcome 1 — 24.0% substantially achieved — the lowest score in the review. Caseworker contact frequency and quality was identified as the most significant problem area (Final Report, p.5–6 and p.41–42).

Well-Being Outcome 3 — 78.7% substantially achieved (Final Report, p.7 and p.50).

6.2.2 Systemic Factor Determinations

In substantial conformity: Statewide Information System, Agency Responsiveness to the Community, Foster and Adoptive Parent Licensing, Recruitment, and Retention (3 of 7).

NOT in substantial conformity: Case Review System, Quality Assurance System, Training, Service Array — the last receiving the lowest possible rating of 1 (Final Report, Table 4, p.14).

6.2.3 Federal Standards Iowa Met in Round 1

Reunification within 12 months: 81.0% (standard: 76.2% or greater).

Adoption within 24 months: 49.0% (standard: 32.0% or greater).

Placement stability: 88.0% (standard: 86.7% or greater).

(Final Report, Table 3, p.13)

6.2.4 Documented Performance Data

Foster care re-entry rate: 25.0% in FY2001 (standard: 8.6% or less); ranged 24.5–26.6% across three prior reporting periods (Final Report, Table 3, p.13; Statewide Assessment, p.103).

Re-abuse rate: 11.2% (standard: 6.1% or less); constant at approximately 10% for multiple

periods. Of repeat maltreatment: 68.5% same or similar circumstances; 58.3% same perpetrator (Final Report, p.20; Statewide Assessment, p.129).

Maltreatment in foster care: 0.89% (standard: 0.57% or less) (Final Report, Table 3, p.13).

Caseworker visits — children (Item 19): 10% Strength rating across 50 cases; 90% Area Needing Improvement. Attributed to "excessively high caseloads" (Final Report, pp.2, 45–46).

Caseworker visits — parents (Item 20): 23% Strength rating; 77% insufficient frequency and quality (Final Report, p.46–47).

Caseloads: 35–55 cases per worker depending on county (Final Report, p.56); average 55 per the state's own staff survey (Statewide Assessment, p.122).

Supervisory ratios: 11:1; most supervisors overseeing approximately 600 cases (Final Report, p.63).

Permanency Outcome 1 by site: 75% Woodbury County; 42% Linn County; 41% Polk County (Final Report, p.4).

Service array: 78 programs closed; approximately 500 private provider staff laid off statewide; long waiting lists even for available services (Final Report, p.9; Statewide Assessment, p.1).

Training: 75% reduction in training budget; training system "not functioning as it should" (Final Report, p.8 and p.64).

Budget: 14% child welfare budget reduction; 116 positions eliminated; 135 held vacant (Statewide Assessment, p.1).

Child fatalities: Of 15 DCDC-reported fatalities, only 10 were actual — 5 were data entry errors; 3 additional fatalities were simultaneously found unreported. 11 of 13 child fatalities in 2001 had no prior DHS involvement (Statewide Assessment, p.111).

Substance use: 42% of a 770-case sample had parental drug involvement (Statewide Assessment, p.2); 48% of the 50 federal cases reviewed cited parental substance abuse as a reason for involvement (Final Report, p.16).

Methamphetamine: Federal stakeholder interviews identified methamphetamine as "a critical problem in the State" and "the primary reason for repeat maltreatment" via chronic neglect. Parents caught in substance abuse treatment waiting lists while ASFA timelines continued to run (Final Report, pp.20, 29, 60).

ICWA non-compliance: Woodbury County courts documented as not complying with ICWA, not allowing Tribal representatives as expert witnesses, and not considering Tribal input. New caseworkers received no ICWA training (Final Report, pp.60, 65).

6.2.4.1 Additional Federal Final Report Findings (Round 1)

Caseworker visit frequency — raw data from case reviews (Final Report, pp.45–47):

Foster care: In 2 of 28 cases, visits occurred monthly. In 26 of 28 cases, visits occurred

LESS THAN MONTHLY.

In-home services: In 1 of 22 cases, visits occurred monthly. In 18 of 22 cases, less than monthly. In 3 of 22 cases, NO VISITS were made.

Visits with fathers: 4 of 27 cases monthly; 20 less than monthly; 3 cases no visits at all.

EXPLANATION: "Caseworkers tend to rely on other forms of contact or information provided by contracted service providers" to substitute for face-to-face contact (Final Report, p.46).

This establishes in Round 1 that proxy monitoring through contracted providers was the documented structural response to overcapacity.

Concurrent planning — documented failure: "Although DHS policy promotes concurrent planning, there was evidence of concurrent planning in only one of the cases reviewed."

(Final Report, p.29) This is a federal finding that out of 28 foster care cases with active court involvement, only 1 demonstrated any evidence of concurrent planning despite DHS policy requiring it within 60 days of removal.

Subsidized guardianship — budget casualty: "Iowa legislation to establish a subsidized guardianship was withdrawn shortly after enactment due to State budget shortfalls. It was expected that such a program would have aided older children in attaining permanency and supported relative placements for children with special needs." (Final Report, p.30)

This is the federal documentation that Iowa's budget cuts eliminated a permanency pathway before it could be implemented — contributing to the documented permanency failures.

Special needs children — foster parent preparedness gap: "Foster parents are not adequately prepared to meet the needs of special needs children (85 percent of the children placed in foster care have special needs)." (Statewide Assessment, foster parent focus group, cited in Final Report, p.67) "Lack of sufficient training is a causal factor to the instability of placements." (Final Report, p.67)

Service delivery — "cookie cutter" assessment: "The same services are being provided to children and parents in a 'cookie cutter' fashion regardless of their appropriateness."

Stakeholders attributed this to elimination of flexible "Decat" funding, and the absence of individualized service capacity (Final Report, p.71).

6.2.5 Federal Sample Methodology

50 cases across three counties; period March 1, 2002 through May 19, 2003. State's own QSR

acknowledged as "not a statistically valid sample" using approximately 160 cases (Final Report, p.15; Statewide Assessment, p.124).

6.2.6 Performance Improvement Plan

Submitted June 22, 2004; approved August 4, 2004.

Federal approval letter from ACF Region VII Regional Administrator Linda K. Lewis, addressed to Director Kevin Concannon.

6.2.6.1 PIP Framework: Better Results for Kids Redesign

The Round 1 PIP is organized under Iowa's "Better Results for Kids" [BR4K] child welfare redesign initiative. This is significant because the PIP is not a standalone corrective action document — it is embedded within a broader system redesign framework that predates the CFSR findings. The PIP uses the BR4K redesign "as the framework for program improvement to address outcomes and systemic factors identified as not in substantial conformity." (PIP, p.4)

AUDIT RELEVANCE: Iowa's own introduction to its PIP acknowledges the systemic failures documented in the CFSR Final Report are attributable to budget cuts: "the statewide assessment and stakeholder interviews during the onsite CFSR attribute many of the current difficulties experienced by DHS to recent budget cuts in all areas of child welfare agency functioning." (PIP, p.4) Iowa simultaneously acknowledged the budget-driven nature of its failures while submitting a PIP that did not include new funding or staffing. The state anticipated continued financial challenges through FY2005 and FY2006. (PIP, p.4)

6.2.6.2 Key PIP Benchmarks and Numerical Goals

The PIP Matrix contains the following negotiated performance goals and baselines:

Safety Outcome 1 (Item 1 — Timeliness of investigations):

Baseline: 73% | Goal: 83% | Midterm: 78% (PIP Matrix, p.1)

Safety Outcome 1 (Item 2 — Repeat maltreatment):

Baseline: 11.4% | Goal: 10.5% | Midterm: 10.95%

(A reduction of less than 1 percentage point from a baseline nearly double the national standard of 6.1% or less) (PIP Matrix, p.2)

Permanency Outcome 1 overall:

Baseline: 50% | Goal: 55% | Midterm: 52.5% (PIP Matrix, p.10)

Item 5 (Foster care re-entries — case review):

Baseline: 60% | Goal: 65% | Midterm: 62.5% (PIP Matrix, p.10-11)

Item 5 (Foster care re-entries — statewide data indicator):

Baseline: 27.7% | Goal: 26.35% | Midterm: 27.0%

(A reduction of 1.35 percentage points from a baseline already three times the national standard of 8.6% or less) (PIP Matrix, p.15)

Item 6 (Stability of foster care placement):

Baseline: 82% | Goal: 87% | Midterm: 84.5% (PIP Matrix, p.16)

Item 7 (Permanency goal for child):

Baseline: 75% | Goal: 80% | Midterm: 77.5% (PIP Matrix, p.20)

Item 9 (Adoption):

Baseline: 55% | Goal: 60% | Midterm: 57.5% (PIP Matrix, p.23)

Item 19 (Caseworker visits with child):

Baseline: 10% | Goal: 25% | Midterm: 17.5%

AUDIT NOTE: A goal of 25% means Iowa set a target where 75% of children would continue to receive inadequate caseworker contact. This is a documented institutionalized acceptance of the caseworker visit failure. (PIP Matrix, p.1 equivalent and PIP p.18)

Family Team Meetings (cross-cutting):

Goal: Conduct Family Team Meetings in 80% of families in the identified target population. (PIP Matrix, pp. 7, 13, 19, 21)

6.2.6.3 Staffing Constraint — Explicit State Admission

"Ideally, DHS would add child welfare case managers and frontline supervisors in order to reduce caseloads and increase capacity/time for workers to conduct meaningful face-to-face visits with children and parents. DHS has not received the funding to add more workers, however, so we have had to develop alternative strategies to address workload and free up worker time for face-to-face visits with children and parents." (PIP, p.17-18)

AUDIT RELEVANCE: The state's primary response to the 90% caseworker visit failure rate was documentation streamlining and caseload diversion — not staffing. Iowa's "alternative strategies" were:

1. Community Care Initiative — diverting low-risk families to community providers
2. Documentation streamlining via contract with the Center for Support of Families
3. Practice standards with monitoring reports (PIP, pp.17-18)

None of these strategies increased caseworker capacity; they attempted to decrease caseworker demand without addressing the underlying resource gap.

6.2.6.4 Judicial Accountability Exclusion

"These strategies will contribute to the success of our PIP but are not included in this PIP at the recommendation of our ACF regional office, since DHS does not have the authority for oversight and achievement of these activities." (PIP, p.5)

The excluded judicial strategies included: establishing a Court Improvement Project task force to address re-entry, permanency, concurrent planning, timely notice to foster parents, preserving connections, and timely judicial decision-making; and Juvenile Court setting performance standards consistent with federal requirements. (PIP, p.5)

AUDIT RELEVANCE: Iowa explicitly excluded court-related accountability measures from the PIP because DHS cannot compel judicial compliance. This is the documented structural gap between the federal PIP accountability mechanism and the court-driven case decisions that the CFSR identified as problematic — including premature TPR, failure to consider kinship, and ICWA non-compliance.

6.2.6.5 Disproportionality Acknowledgment

The PIP explicitly documents racial disproportionality in the Iowa child welfare system:

"Black and Native American children are twice as likely to be victims of child abuse. Black children are almost four times as likely to be placed in foster care, while Native American children are over five times as likely to be placed in foster care." (PIP, p.6)

Additionally, the PIP documents that DHS received 8 specialized methamphetamine specialist positions statewide to address substance abuse — "Methamphetamine use is at an all time high." (PIP, p.21)

6.2.6.6 Methamphetamine-Specific Response

The PIP established a dedicated "Meth Specialist" program:

"Establish specialized substance abuse positions, Meth Specialists, for each judicial district to provide direct service in reduced caseloads, consultation, and training to front-line workers." (PIP Matrix, p.8)

Training specifically included: "Tough Problems, Tough Choices: Using Guidelines in Substance Abuse Cases [focus on meth abuse]" delivered statewide via ICN and phone conferencing. (PIP Appendix IV, p.49)

AUDIT RELEVANCE: Iowa's Round 1 PIP explicitly acknowledged methamphetamine as a critical driver of child welfare involvement and created a specialized meth specialist infrastructure at the judicial district level. This infrastructure was in place when Iowa went into the Round 2 CFSR, yet substance abuse services remained the most consistently cited systemic

gap in Round 2 and Round 3 — and the CIA Report (2023) and CWPPG Provider Forums (2019) independently confirmed it remained unresolved twenty years later.

6.2.6.7 Financing Acknowledgment — IV-E Structural Constraint

The PIP explicitly documents the structural fiscal limitations that constrained the state's response — not just the budget cuts, but the federal funding architecture itself:

"Federal Title IV-E funding only reimburses expenditures associated with out-of-home placement. Activities to prevent placement or to return a child home more quickly...are ineligible for federal IVE funding."

"Federal IVE funds can only be used on maintenance (i.e., 'room and board') costs.

Activities to actually address a child's service needs or the problems that led to out-of-home placement are not eligible for IVE funding."

"One other issue/barrier...both pay for activities, not outcomes." (PIP, pp.19-20)

AUDIT RELEVANCE: Iowa's Round 1 PIP explicitly identified the federal IV-E funding structure as a barrier to the prevention-oriented practice required to address the documented systemic failures. This is the Round 1 state-admitted documentation of the structural funding inversion that the Sivic Solutions Report (2023) quantified 20 years later as \$12.5 million in unclaimed federal prevention reimbursement (Section 10.7).

6.2.6.8 Additional PIP Matrix Benchmarks — Complete Item Coverage (Pages 100–145)

Item 14 (Preserving connections): Baseline: 79% | Goal: 84% | Midterm: 81.5%

Item 15 (Relative placement): Baseline: 77% | Goal: 82%

Item 16 (Relationship of child with parents): Baseline: 79% | Goal: 84% | Midterm: 81.5%

Item 18 (Child/family involvement in case planning): Baseline: 66% | Goal: 71% | Midterm: 68.5%

Item 19 (Worker visits with child): Baseline: 10% | Goal: 25% | Midterm: 17.5%

Item 20 (Worker visits with parents): Baseline: 23% | Goal: 30% | Midterm: 26.5%

Well-Being Outcome 1 overall: Baseline: 24% | Goal: 29% | Midterm: 26.5%

Well-Being Outcome 3 (physical and mental health): Baseline: 78.7% | Goal: 83.7% | Midterm: 81.2%
(PIP Matrix, pp.28–43)

6.2.6.9 ICWA — Time-Bound Commitments Made in 2004, Unfulfilled by 2018

Items 14.4–14.5 of the PIP Matrix contain explicit ICWA benchmarks including:

training for DHS, attorneys, and judges (Q1); ICWA policy manual letter (Q1); initial ICWA compliance review (Q2); manual revision based on review (Q4); and at least one tribal MOU by Q5. (PIP Matrix, pp.28–29)

Round 3 Final Report (2018) continued to document the same deficiency: three of 13 applicable

cases failed specifically due to "no ICWA notice to the Tribe or follow-up" — a 14-year gap.

6.2.6.10 Kinship Care — Deferred to Late PIP Quarters, Gap Persisted 19 Years

Kinship care policy guide and training were assigned to Q5–Q6 — among the last action steps. (PIP Matrix, pp.29–30) Sivic Solutions (2023) found Iowa's \$3.3M kinship navigator program still not reimbursable because it had never been submitted to the federal Clearinghouse — 19 years after Round 1 identified kinship capacity as a systemic need.

6.2.6.11 Flexible Service Funds — Item 37

Item 37 proposed increasing wraparound and flexible funds for individualized services, with rules effective by Q4. (PIP Matrix, p.62) C!A (2023) documented the same gap: "family preservation funds are too limited and not flexible enough." Iowa identified this need in 2004; it remained unresolved through all three subsequent CFSR rounds and independent evaluations.

6.3 CFSR ROUND 2 (2010–2011)

6.3.1 Context and Period Under Review

The Round 2 federal onsite review was conducted the week of August 23, 2010, with the Executive Summary issued January 2011 and the Full Final Report issued February 2011. The period under review was April 1, 2009 through August 27, 2010. The State Data Profile covered the 12-month CFSR target period ending September 30, 2008 (Final Report, p.1).

Case sample: 65 cases (40 foster care, 25 in-home services) across three sites — 17 cases in Linn County, 31 in Polk County, 17 in Webster County (Final Report, p.1 and p.25).

NOTE: The federal report explicitly states that Round 2 performance is NOT directly comparable to Round 1 due to changes in sample size (50 to 65), case stratification, and revised item criteria. This is a federal acknowledgment of methodological discontinuity (Final Report, pp.1–2 and p.27).

6.3.2 Workforce Context at Time of Review

Between rounds, Iowa had improved workforce capacity from 2005 to 2008: supervisory ratios reduced from 11:1 to 7:1 by adding 23 clinical consultant/supervisors; caseloads reduced from 51 to 30 per worker by adding 20 child welfare caseworkers and reassigning 33 others. However, across-the-board budget reductions (1.5% in December 2008; 10% in November 2009) and the voluntary retirement of 638 IDHS staff effective June 24, 2010, reversed those gains.

"Supervisory ratios and social worker caseloads have risen and are expected to continue to rise"

at the time of the review (Statewide Assessment, p.10 and p.40).

6.3.3 Outcome Determinations — Round 2

Iowa did not achieve substantial conformity with any of the seven CFSR outcomes in the 2010 CFSR. This is the first federal review in which Iowa failed all seven primary outcomes (Final Report, Executive Summary, p.3).

The federal report noted three individual items as Strengths:

- Item 11: Proximity of placement (96% Strength)
- Item 22: Physical health of child (94% Strength)
- Item 23: Mental/behavioral health of child (91% Strength)

(Final Report, Table 2, p.20)

Outcome-level findings (Final Report, Tables 1 and 2, pp.19–20):

Safety Outcome 1 — 77.8% of 27 applicable cases substantially achieved (standard: 95%).

Iowa did not meet the national standard for absence of maltreatment recurrence (91.9% vs.

94.6% standard). Iowa did meet the standard for absence of maltreatment in foster care.

Safety Outcome 2 — 63.1% substantially achieved (standard: 95%). NOTE: Iowa was in substantial conformity with Safety Outcome 2 in Round 1 (93.5%). Round 2 represents a documented decline from conformity to non-conformity. Key concerns: services did not address safety concerns; lack of ongoing safety and risk assessments throughout the life of the case.

Permanency Outcome 1 — 37.5% substantially achieved (standard: 95%). By site: 0% Linn County; 65% Polk County; 20% Webster County. Iowa did not meet national standards for timeliness and permanency of reunification or placement stability.

Permanency Outcome 2 — 75.0% substantially achieved (standard: 95%).

Well-Being Outcome 1 — 40.0% substantially achieved (standard: 95%). By case type: 45% foster care; 32% in-home services.

Well-Being Outcome 2 — 93.0% substantially achieved (standard: 95%). Close to the threshold but below the raised 95% standard applied in Round 2.

Well-Being Outcome 3 — 88.1% substantially achieved (standard: 95%). Despite individual items rated as Strengths, combined case performance failed to meet the threshold.

6.3.4 Systemic Factor Determinations — Round 2

In substantial conformity (5 of 7): Statewide Information System (score 4), Case Review System (score 3), Staff and Provider Training (score 3), Agency Responsiveness to the Community (score 4), Foster and Adoptive Parent Licensing, Recruitment, and Retention

(score 3) (Final Report, Table 3, pp.21–22).

NOT in substantial conformity (2 of 7):

Quality Assurance System (score 2): The federal review found it unclear whether the QA system effectively addresses key practice areas, provides feedback on findings, or evaluates program improvement measures — due in part to DHS restructuring occurring during the review period (Final Report, p.14).

Service Array and Resource Development (score 2): Services not accessible in all jurisdictions; lack of culturally sensitive services; limitations of mental health coverage affecting individualized service delivery (Final Report, p.16).

6.3.5 The QA Self-Assessment Discrepancy — Critical Federal Finding

The 2010 Final Report explicitly flags a discrepancy between the state's own QA ratings and the federal case review findings as a systemic concern:

"An additional concern noted in the CFSR pertained to the State's own quality assurance (QA) rating for the items as reported in the Statewide Assessment. For some items, these ratings were high and differed considerably from the 2010 CFSR case review findings. This discrepancy raises questions regarding the effectiveness of the State's QA case review system in identifying the strengths and needs of the service delivery system, and the capability of the QA system to evaluate program improvement measures accurately."

(Final Report, Executive Summary, p.4)

The Round 2 PIP independently confirms the source of this discrepancy: "Iowa's use of the CFSR case reading tool resulted in higher scoring in most categories than found during the 2010 federal on-site review. Two factors appeared related. One was training and one was scoring oversight. Training 70 plus supervisors in the nuance of the CFSR tool, providing an adequate number of mentored case reading experiences, and feedback, required resources in both time and personnel beyond what was available in Iowa. While reviewers were asked to use the CFSR criteria, only the item scoring was captured on Iowa's automated tool, therefore the sub-item completion and scoring could not be monitored or validated." (PIP, p.32)

State self-reported case reading performance (as of December 2009) vs. federal case review findings at the time of the Round 2 CFSR:

Face-to-face visits with children:

State case reading data — 88%; administrative data — 81%; federal finding — 65% Strength.

A 23-percentage-point gap between the state's own case reading figure and the federal finding.

Face-to-face visits with parents:

State case reading data — 71%; administrative data — 45%; federal finding — 43% Strength.

(Statewide Assessment, File 2, p.141; Final Report, Table 2, p.20)

6.3.6 Item-Level Performance Data

From Final Report Tables 1 and 2 (pp.19–20):

Item 17 — Needs and services of child, parents, foster parents: 45% Strength (ANI).

Item 18 — Child/family involvement in case planning: 49% Strength (ANI).

Item 19 — Caseworker visits with child: 65% Strength (ANI).

Item 20 — Caseworker visits with parents: 43% Strength (ANI).

Item 5 — Foster care reentry: 80% Strength (ANI).

Item 6 — Stability of foster care placements: 62.5% Strength (ANI).

Item 7 — Permanency goal for child: 64% Strength (ANI).

Item 8 — Reunification, guardianship, placement with relatives: 64% Strength (ANI).

Item 9 — Adoption: 59% Strength (ANI).

Item 10 — Other planned living arrangement: 50% Strength (ANI).

Item 11 — Proximity of placement: 96% Strength (STRENGTH).

Item 13 — Visiting with parents and siblings: 66% Strength (ANI).

Item 14 — Preserving connections: 82% Strength (ANI).

Item 15 — Relative placement: 76% Strength (ANI).

Item 16 — Relationship with parents: 67% Strength (ANI).

6.3.7 Substance Use — Federal Case Sample

Of the 65 cases reviewed, parental substance abuse was the primary reason for case opening in 17 of 40 foster care cases (42.5%) — the single most common primary reason for foster care case opening in the Round 2 sample (Final Report, p.25).

6.3.8 Round 1 PIP Completion Status — Federal Acknowledgment

The Round 2 Final Report explicitly states that Iowa "met its goals" for each outcome and systemic factor by the end of the Round 1 PIP implementation period (Final Report, pp.5–12).

This creates a documented structural paradox: Iowa was certified as having completed the Round 1 PIP, then proceeded to fail all seven outcomes in Round 2. The PIP mechanism produced documented goal completion without durable systemic improvement.

6.3.9 Federal Sample Methodology

65 cases, stratified across three counties, period April 1, 2009 through August 27, 2010. The increase from 50 to 65 cases and the revised 95% conformity threshold (vs. 90% in Round 1)

make direct numeric comparison with Round 1 unreliable, as explicitly noted by the federal review (Final Report, pp.1–2).

6.3.10 Round 2 Performance Improvement Plan

The Round 2 PIP was submitted September 2, 2011. The PIP was organized under four themes: Family Engagement, Permanency Planning, Services Provision, and Quality Assurance (PIP, p.8–9).

PIP four primary themes (PIP Narrative, pp.8–9):

- Family Engagement: Increase quality and frequency of caseworker visits with parents and children; expand Responsible Fatherhood and Non-Custodial Parent initiative; expand Parent Partners program.
- Permanency Planning: Increase effective use of Family Team Decision-Making Meetings; implement Plan, Do, Study, Act (PDSA) model for placement stability; improve Family Interaction practice; improve permanency for youth through Permanency Roundtables.
- Services Provision: Strengthen Community Partnership for Protecting Children; align service contracts with safety, permanency, and well-being outcomes; expand Joint Substance Abuse Protocol to additional counties; develop children's mental health services capacity.
- Quality Assurance: Implement a reliable and valid QA system for case reviews using the federal CFSR Onsite Review Instrument; implement a model of supervisory practice; implement Results Oriented Management (ROM) reporting system.

Caseworker visit benchmarks — Round 2 PIP: The PIP explicitly acknowledges the baseline figures from both rounds: "In the 2003 CFSR, 10% of cases reviewed substantially achieved Item 19, which is caseworker visits with children, and 23% of cases reviewed substantially achieved Item 20, which is caseworker visits with parents. By contrast, in Iowa's 2010 CFSR, 65% of cases reviewed substantially achieved Item 19 and 43% of cases reviewed substantially achieved Item 20. Iowa made a significant improvement between the 2003 and 2010 CFSRs, especially for caseworker visits with children." The PIP identifies "high caseloads" as a continuing barrier to achieving frequency of visits. (PIP, p.12–13)

Round 2 PIP — improvement goals for monitored items (PIP Matrix, Part C, pp.87–91):

The Round 2 PIP documents performance as measured in the Final Report for each item subject to monitoring, and establishes that negotiated improvement goals are "to be determined based on data from the first four quarters of PIP." This means no specific numerical improvement target was negotiated at submission — a structural accountability gap carried forward from Round 1. Baseline figures from the Final Report as documented in the PIP Matrix are:

Item 1 (timeliness of investigations): 85%

Item 3 (services to protect children): 77%

Item 4 (risk of harm): 65%

Item 7 (permanency goal): 64%

Item 10 (other planned living arrangement): 50%

Item 17 (needs and services): 45%

Item 18 (child/family involvement in case planning): 49%

Item 19 (caseworker visits with child): 65%

Item 20 (caseworker visits with parents): 43%

Placement Stability national standard: Iowa's performance at Final Report was 94.0 (standard

101.5); negotiated improvement goal set at 96.1 (PIP Matrix, Part B, p.86).

Round 2 PIP — QA system reform: The PIP explicitly acknowledges that the QA data provided in the Statewide Assessment "was not representative of what was found during the on-site review." Iowa committed to establishing new baselines using the federal CFSR Onsite Review Instrument rather than its own case reading tool, with the baseline period defined as October 2011 to September 2012 — over a year after the federal review (PIP, p.36).

Round 2 PIP — substance abuse protocol: The PIP documents expansion of the Joint Substance Abuse Protocol (HF2310) to additional counties, acknowledging that the child welfare and substance abuse treatment systems historically operated without coordination such that "a family could be involved with all three agencies and 'no one would know that.'" (PIP, p.19–20)

Round 2 PIP — accountability provisions: No individual or institutional consequences were defined for missing benchmarks. Accountability was framed around the Service Business Team's review and approval authority. The "evidence of completion" standard for most benchmarks was a written summary or implementation plan — process indicators, not outcome measures (PIP Matrix, throughout).

Round 2 PIP — placement stability finding: The PIP acknowledged that Iowa did not meet the national standard for Permanency Composite 4 (Placement Stability) and that the 2010 onsite review identified that "initial placements were based on bed availability, not the skills and training of foster parents" — a direct state acknowledgment of placement matching failure (PIP, p.10).

6.4 CFSR ROUND 3 (2018–2019)

[Source-verified: Statewide Assessment (pp.1–149, February 15, 2018)]

[Source-verified: Federal Final Report (2018)]

[Round 3 PIP: PENDING]

6.4.1 Context and Period Under Review

The Round 3 Statewide Assessment was submitted February 15, 2018. The CFSR sample period was April 1, 2017 through September 30, 2017; the full Case Review Period Under Review extended to April 1, 2017 through September 30, 2018 (Statewide Assessment, p.4).

The period under review for national performance indicators was October 1, 2014 through September 30, 2016 (AFCARS) and October 1, 2014 through September 30, 2016 (NCANDS).

Important methodology note: The federal Children's Bureau discovered issues with the syntax for the national safety and permanency performance indicators for Round 3. Due to these issues, the Children's Bureau advised states that the national indicators were "to be used for contextual information only for CFSR Round 3." Assessment of substantial conformity with safety, permanency, and well-being outcomes was determined exclusively by case reviews conducted during the federal onsite review (Statewide Assessment, p.11). This means the national data standards were suspended as a conformity determinant for Round 3.

An independent review of Iowa's child welfare system was conducted by the Child Welfare Policy and Practice Group (CWPPG), a nonprofit technical assistance organization with experience in evaluations in more than two dozen states. CWPPG focused on two of Iowa's six Service Areas (Des Moines and Cedar Rapids). CWPPG's report is incorporated throughout the Statewide Assessment as an independent source (Statewide Assessment, p.11).

6.4.2 Workforce Context at Time of Review — Self-Documented

The Round 3 Statewide Assessment contains a state self-admission on workforce trajectory that appears in multiple sections:

"Iowa's workforce has not grown at the pace caseload has grown, with current caseloads the largest since 2010 and current workforce the smallest and likely to shrink further."
(Statewide Assessment, pp.50, 75)

This statement is repeated in the assessment for multiple CFSR items as an attributed barrier, establishing it as a documented systemic condition rather than an isolated observation.

Additionally, in the Safety section: "the relatively stable size of the workforce performing child protective assessments (average 2% growth in last two years) in Iowa while the workload, the number of assessments, has grown more quickly (average 20% growth in last two years)." (Statewide Assessment, p.34)

The DHS reduced its internal case review sample from 150 cases (SFY 2016) to 65 cases

(SFY 2017) because reviewing 150 cases was documented as "unsustainable with resources available." The reduction is explicitly noted as creating a statistical discontinuity: "due to the difference in sample size, comparisons cannot be made between SFY 2016 and SFY 2017 case reviews." (Statewide Assessment, p.33)

6.4.3 National Performance Indicators — Contextual Data

The following figures are drawn from the State Data Profile dated September 2017 and are provided as contextual data only, not as conformity determinants (Statewide Assessment, pp.14, 22):

Recurrence of maltreatment (FFY 2015–2016): Iowa — 14.1%; national performance standard — 9.5% or less. Iowa exceeded the national standard by approximately 50%.

Maltreatment in foster care (FFY 2015): Iowa — 19.77 victimizations per 100,000 days in foster care; national standard — 9.67 or less. Iowa exceeded the national standard by more than double.

Both figures are risk-standardized (marked "***" in the source).

6.4.4 State Self-Assessment Case Review Data — SFY 2016 and SFY 2017

The following case review data are drawn from Iowa's internal case reviews conducted under the federal CFSR Onsite Review Instrument. All data are state self-reported and subject to the same QA limitations documented in Rounds 1 and 2.

Safety Outcome 1 (Item 1 — Timeliness of investigations):

SFY 2016 (N=150): 86% (n=67/78)

SFY 2017 (N=65): 92% (n=24/26); goal was 95% or more (Statewide Assessment, pp.33–34)

Safety Outcome 2:

Item 2 (Services to prevent removal): SFY 2016: 91%; SFY 2017: 81% (n=17/21)

Item 3 (Risk and Safety Assessment and Management): SFY 2017: 62% (n=40/65); goal 95%

By case type for Item 3: Foster Care 68%; In-Home Services 48% (Statewide Assessment, p.35)

Permanency Outcome 1:

Item 4 (Placement stability): SFY 2017: 85%

Item 6 (Timely achievement of permanency goals): 12 cases ANI; concurrent planning

"has been a barrier to Iowa's performance for this item for several years" (Statewide Assessment, p.42)

Permanency Outcome 2:

Item 7 (Placement with siblings): SFY 2016: 84%; SFY 2017: 100% (n=19/19) — met 95%

Item 8 (Visiting with parents and siblings): SFY 2016: 66%; SFY 2017: 71% (n=20/28)

Item 9 (Preserving connections): SFY 2016: 82%; SFY 2017: 66% (n=25/38)

Item 10 (Relative placement): SFY 2016: 68%; SFY 2017: 81% (n=25/31)

Item 11 (Relationship of child with parents): SFY 2016: 63%; SFY 2017: 56% (n=15/27)

(Statewide Assessment, pp.42–43)

Well-Being Outcome 1:

Item 12 (Needs and services): SFY 2016: 51%; SFY 2017: 46% (n=30/65)

Item 13 (Child/family involvement in case planning): SFY 2016: 54%; SFY 2017: 54% (n=34/63)

Item 14 (Caseworker visits with child): SFY 2016: 45%; SFY 2017: 65% (n=42/65)

Item 15 (Caseworker visits with parents): SFY 2016: 21%; SFY 2017: 24% (n=13/55)

(Statewide Assessment, p.45)

Caseworker visits — parent sub-item analysis (SFY 2017):

Item 15A (Visits with mother — Frequency): 53%; (Quality): 62%

Item 15B (Visits with father — Frequency): 26%; (Quality): 28%

(Statewide Assessment, p.47)

Well-Being Outcome 3:

Item 17 (Physical health): SFY 2016: 54%; SFY 2017: 67% (n=29/43)

Item 18 (Mental/behavioral health): SFY 2016: 57%; SFY 2017: 57% (n=29/51)

(Statewide Assessment, p.52)

6.4.5 Caseworker Visit Narrative — Workforce Attribution

The Statewide Assessment includes a direct narrative attribution connecting caseworker visit deficiencies to the workforce/workload gap:

"Iowa continues to see gradual improvement of approximately 2% per year for item 14 in the frequency of caseworker visits with children, now at 85% seen each month, and greater improvement in quality of caseworker visits with child from 45% in SFY 2016 to nearly 65% in SFY 2017. However, practice in engaging parents, especially fathers, continues to be an ANI for items 12, 13, and 15, particularly in foster care cases versus in-home cases when both parents are more likely to be applicable for rating in these items. The work with fathers, specifically non-custodial fathers which is the most challenging, is to some degree a practice focus and skill, but also definitely challenging in terms of logistics as Iowa's workforce has not grown at the pace caseload has grown, with current caseloads the largest since 2010 and current workforce the smallest and likely to shrink further."

(Statewide Assessment, p.50)

6.4.6 Drug-Involved Children — No Protocol Admission

The Statewide Assessment contains a critical self-admission regarding the absence of a standard protocol for a defined high-risk population:

"One ad-hoc report/analysis project identified and quantified a set of factors in common across Recurrence of Maltreatment, Maltreatment in Care, and Re-entry into Foster Care. While it is probably common practice knowledge that the three factors contribute significantly to each of the measures, examining the three together helped Iowa to identify that we had no protocol (standard or best practice) when young children, who first experience the child welfare system while under 6, are abused or neglected and removed due to parental drug use."

The analysis further documents: "Not only does this represent about half of Recurrence, it is also nearly half of abuse in care. The abuse in care is not happening at the hands of substitute caregivers, but during weekend visits with the family during placement and over the six months while on trial home visits (THVs). Additionally, the frequency of the incidents of children returned home continues beyond the six months of THV and then begins to contribute to nearly half of the Re-entry into Foster Care for the young child's second episode."

(Statewide Assessment, p.85)

Quantitative data supporting this admission (Statewide Assessment, p.86):

Of 134 children under age 6 with documented abuse during a foster care episode:

— 81 (60%) were removed due to parental drug use.

Of those 81 drug-removal, under-6 children with abuse in care:

— 74 (91%) were experiencing their first episode in foster care.

Of re-entries into foster care during SFY 2017:

— 518 re-entries involved parental drug use as removal reason.

— 79 of 134 total re-entries (59%) involved parental drug use.

6.4.7 Substance Use — Drug Endangered Children Legislative Action

Senate File 2258 (2016 Iowa Legislative Session) convened a Drug Endangered Children (DEC) Workgroup. The workgroup documented stakeholder concerns that substance abuse cases were being assigned to the voluntary Family Assessment pathway rather than the Child Abuse Assessment pathway, and that Iowa lacked mandatory reporting requirements for infants affected by prenatal drug or alcohol use. Three of four workgroup recommendations were passed by both chambers and signed into law on April 20, 2017, including modifications to expand mandatory reporting and to require meth, cocaine, heroin, and synthetic opioids to be assessed on the Child Abuse Assessment pathway regardless of the child's age (Statewide Assessment, File 2, pp.23–25).

6.4.8 Quality Assurance Self-Assessment — Critical Finding

Iowa rated the Quality Assurance Systemic Factor in substantial conformity, stating: "Iowa rates this item as a strength because Iowa's quality assurance system: operates statewide... has standards to evaluate the quality of services...identifies the strengths and needs of the service delivery system...provides relevant reports...evaluates implemented measures." (Statewide Assessment, p.90)

Iowa's self-rating of QA as a "strength" occurred simultaneously with the documented admission that:

- The case review sample had been reduced by 56% (from 150 to 65 cases) because the higher volume was "unsustainable with resources available."

- Iowa had identified that it had "no protocol (standard or best practice)" for a defined high-risk population representing approximately half of all recurrence, maltreatment in care, and re-entry incidents.

- Iowa's communication process for sharing QA findings with stakeholders was documented as "much of this is informal rather than a systematic process." (Statewide Assessment, p.78)

These concurrent admissions occurred within the same document that rated the QA system as a strength. The federal Final Report determination on this item is pending upload.

6.4.9 ICWA and Tribal Consultation

The 2017 Child Welfare Stakeholders Survey included "Native American Tribe" as a community partner category. Of 128 respondents, 0% identified as Native American Tribe (Statewide Assessment, File 2, p.113).

ICWA non-compliance was documented in SFY 2017 case reviews: three of 13 cases rated ANI for Item 9 (Preserving Connections) were due to "no ICWA notice to the Tribe or follow-up" (Statewide Assessment, p.44).

The Meskwaki Family Services continued to be listed as a stakeholder participant through the Community Initiative for Native Children and Families (CINCF), representing continuity from Round 2 tribal engagement structures (Statewide Assessment, File 2, p.120).

6.4.10 CWPPG Independent Review Findings

The Child Welfare Policy and Practice Group (CWPPG) conducted an independent targeted review of two service areas (Des Moines and Cedar Rapids). Key documented findings include:

On placement resources: "DHS staff encounters difficulty finding suitable placements from among the available families and some of those interviewed expressed the belief that there are

many families who are unable or unwilling to provide the quality of care that children require." (CWPPG, p.17, cited in Statewide Assessment, p.41)

On service quality: "...the quality and consistency of services, especially those offered through FSRP, is questionable... The qualifications of staff, in accordance with the contracts reviewed, do not seem commensurate with the expectations outlined." (CWPPG, p.24, cited in Statewide Assessment, p.89)

On concurrent planning: "Several stakeholders raised a concern that concurrent planning was not consistently implemented effectively." (CWPPG, cited in Statewide Assessment, p.41)

On Community Care referrals: DHS is making "cold" referrals to Community Care — families referred "whether or not they have committed to be voluntarily involved in a plan of services and there is no follow-up to determine the family's outcome." Providers are paid \$500 per referral "whether or not a family actually engages in services." (CWPPG, pp.11–12, cited in Statewide Assessment, p.89)

On FSRP staff qualifications and turnover: "A consistent theme in interviews conducted during this review was that FSRP services staff were not well qualified for the level of the work they were expected to do...staff turnover 'ebbs and flows' in relationship to DHS hiring as many personnel leave positions in contracted agencies for better pay and benefits at DHS." (CWPPG, cited in Statewide Assessment, p.89)

6.4.11 State Self-Assessment — Systemic Factor Ratings (Iowa's Own Determinations)

The following represents Iowa's own self-assessment ratings, not the federal determination (which requires the Final Report):

Statewide Information System: Strength (JARVIS/FACS/STAR system)

Case Review System: Multiple items — strength and ANI mix

Quality Assurance System: Iowa self-rated as STRENGTH (contested above)

Staff and Provider Training: Iowa rates NOT in substantial conformity — all three items rated ANI, including only 49% of ongoing staff completing required 24 hours of training in SFY 2017 (Statewide Assessment, File 2, p.3 and p.10)

Service Array and Resource Development: Iowa rates NOT in substantial conformity — both items rated ANI; top barriers: mental health services availability (18%), transportation (15%), high caseworker caseloads (10%) (Statewide Assessment, File 2, p.14 and p.18)

Agency Responsiveness to the Community: Iowa rates in substantial conformity — both items rated strengths (Statewide Assessment, File 2, p.41)

Foster and Adoptive Parent Licensing: Iowa rates multiple items as strengths; diligent

recruitment of diverse families rated ANI (Statewide Assessment, File 2, pp.140–149)

6.4.12 Federal Sample Methodology

The federal onsite review sample period for Round 3 was April 1, 2017 through September 30, 2018. The case sample size was 65 cases, unchanged from Round 2. Iowa's own internal review sample had been reduced from 150 to 65 cases during the same period, creating a statistical comparability concern between SFY 2016 and SFY 2017 internal data (Statewide Assessment, p.33).

6.4.13 Federal Final Report — Outcome and Systemic Factor Determinations

The Round 3 federal onsite review was conducted via a State Conducted Case Review process in five service areas (Cedar Rapids, Des Moines, and Eastern, Northern, and Western Iowa) between April 1, 2018 and September 30, 2018. The case sample consisted of 65 cases (40 foster care, 25 in-home services), with one case classified as in-home alternative/differential response (Final Report, p.1).

HEADLINE FINDING: None of the 7 outcomes was found to be in substantial conformity.

Only 2 of the 7 systemic factors were found to be in substantial conformity (Final Report, p.3).

This represents the third consecutive full-cycle federal review in which Iowa failed all seven primary outcomes, and the second consecutive review in which Iowa failed 5 of 7 systemic factors. Iowa's systemic factor performance deteriorated from Round 2 (5 of 7 in conformity) to Round 3 (2 of 7 in conformity) — a structural regression across the systemic infrastructure of the child welfare system.

Outcome-Level Conformity Determinations (Final Report, Appendix A, pp.A-1 to A-3):

Safety Outcome 1 — Not in substantial conformity: 71% substantially achieved (35 applicable cases). Item 1 (timeliness of investigations): 71% Strength — ANI.

Safety Outcome 2 — Not in substantial conformity: 51% substantially achieved (65 cases).

By case type: 55% foster care; 46% in-home; 0% differential response.

Item 2 (services to protect children): 86% Strength — ANI.

Item 3 (risk and safety assessment and management): 51% Strength — ANI.

Permanency Outcome 1 — Not in substantial conformity: 45% substantially achieved (40 applicable cases).

Item 4 (stability of foster care placement): 80% Strength — ANI.

Item 5 (permanency goal for child): 85% Strength — ANI.

Item 6 (achieving reunification/guardianship/adoption): 60% Strength — ANI.

Permanency Outcome 2 — Not in substantial conformity: 68% substantially achieved (40 applicable cases).

Item 7 (placement with siblings): 88% Strength — ANI.

Item 8 (visiting with parents and siblings): 74% Strength — ANI.

— Visitation with mothers (frequency and quality): 82% of 28 applicable cases

— Visitation with fathers (frequency and quality): 63% of 16 applicable cases

Item 9 (preserving connections): 63% Strength — ANI.

Item 10 (relative placement): 78% Strength — ANI.

Item 11 (relationship of child with parents): 66% Strength — ANI.

— Concerted efforts for mother: 78% of 27 applicable cases

— Concerted efforts for father: 50% of 16 applicable cases

Well-Being Outcome 1 — Not in substantial conformity: 38% substantially achieved (65 cases).

By case type: 48% foster care; 25% in-home; 0% differential response.

Item 12 (needs and services): 45% Strength — ANI.

Sub-item 12A (needs and services to children): 66% Strength — ANI.

Sub-item 12B (needs and services to parents): 44% Strength — ANI.

— Concerted efforts for mothers: 59% of 58 applicable cases

— Concerted efforts for fathers: 45% of 51 applicable cases

Sub-item 12C (needs and services to foster parents): 85% Strength — ANI.

Item 13 (child and family involvement in case planning): 49% Strength — ANI.

— Children involved: 65% of 40 applicable cases

— Mothers involved: 64% of 58 applicable cases

— Fathers involved: 50% of 50 applicable cases

Item 14 (caseworker visits with child): 51% Strength — ANI.

By case type: 60% foster care; 38% in-home; 0% differential response.

Item 15 (caseworker visits with parents): 25% Strength — ANI.

By case type: 32% foster care; 17% in-home; 0% differential response.

— Sufficient frequency and quality with mothers: 43% of 58 applicable cases

— Sufficient frequency and quality with fathers: 44% of 50 applicable cases

Well-Being Outcome 2 — Not in substantial conformity: 84% substantially achieved (43 applicable cases).

Item 16 (educational needs of child): 84% Strength — ANI.

NOTE: The 95% standard applies to this item as the sole item for this outcome. Performance of 84% fails the 95% threshold despite being the highest-performing outcome.

Well-Being Outcome 3 — Not in substantial conformity: 48% substantially achieved (61

applicable cases).

By case type: 53% foster care; 35% in-home; 100% differential response.

Item 17 (physical health): 59% Strength — ANI.

Item 18 (mental/behavioral health): 56% Strength — ANI.

(Final Report, Sections II and Appendix A)

6.4.14 Federal Final Report — Systemic Factor Determinations

In substantial conformity (2 of 7):

— Agency Responsiveness to the Community: Both items rated Strength.

— Foster and Adoptive Parent Licensing, Recruitment, and Retention: 3 of 4 items Strength.

NOT in substantial conformity (5 of 7):

— Statewide Information System (Item 19 — ANI): Iowa's information system is "not able to accurately and readily identify the location of children in foster care." Stakeholders identified lack of policy expectations for timely documentation of placement changes (Final Report, p.15).

— Case Review System: ALL 5 items rated ANI — zero Strengths in this factor.

Item 20 (written case plan): Case plans not consistently developed jointly with parents; disparate practice between mothers and fathers; often created without parental knowledge.

Item 21 (periodic reviews): Foster Care Review Boards and Administrative Reviews not occurring consistently across state; no reliable tracking system.

Item 22 (permanency hearings): Initial permanency hearings combined with contested TPR hearings or delayed beyond 12-month requirement; inconsistencies in how hearings are coded.

Item 23 (TPR filing): TPR petitions not routinely filed in a timely manner; barriers include high caseworker caseloads and limited county attorney resources.

Item 24 (notice to caregivers): Caregivers do not consistently receive notices; relative caregivers informed of right to be heard but foster/adoptive caregiver notices do not consistently convey this right.

(Final Report, pp.16–17)

— Quality Assurance System (Item 25 — ANI): Elements of a QA system exist but "not all elements are functioning as required in all geographic areas." "There is not a functional feedback loop to ensure that frontline staff and providers can identify strengths and needs." Program improvement measures unclear across the state. Frontline staff either lack access to relevant data reports or do not utilize them (Final Report, p.18).

NOTE: Iowa's self-assessment rated QA as a strength; the federal determination was ANI.

This replicates the Round 2 QA discrepancy pattern.

— Staff and Provider Training: ALL 3 items rated ANI — zero Strengths in this factor.

Item 26 (initial staff training): Very few new staff complete initial training within required timeframes; significant waiting times for cyclical training; varied stakeholder views on effectiveness.

Item 27 (ongoing staff training): Many staff across the state do not complete required ongoing training hours; lack of supervisor training for leadership and management development.

Item 28 (foster and adoptive parent training): No data provided in statewide assessment to determine whether foster parents and state-licensed facility staff completed required training; training does not adequately prepare caregivers for the "increasingly high-need population of children entering foster care."

(Final Report, pp.18–19)

— Service Array and Resource Development: BOTH items rated ANI — zero Strengths.

Item 29 (array of services): While services exist, "significant issues of accessibility" in rural areas; barriers include transportation, distance, and waitlists for mental health, substance abuse treatment, disability services, and housing.

Item 30 (individualizing services): Variation in tailoring services; urban/rural disparity; lack of foster homes affects capacity to match placements to child needs; unmet need for post-adoption services.

(Final Report, p.20)

6.4.15 Children's Bureau Cross-Cutting Observations

The Federal Final Report contains a set of cross-cutting observations from the Children's Bureau that are of direct evidentiary relevance:

On drug testing and substance abuse services: "A lack of drug testing locations and services to get parents to those locations was also a statewide concern, as were limited drug testing hours that can be a barrier for parents employed during traditional work hours. Stakeholders indicated the possible overuse of drug testing in Iowa, such as even when behavioral indicators of substance abuse are not present." (Final Report, p.4)

On placement matching: "Youth are placed in homes or facilities based on bed availability or location rather than on services that match the youth's needs." This is the federal auditors directly echoing the placement instability finding first documented in Round 2 (Final Report, p.4).

On workforce: "According to the statewide assessment, caseload sizes are the largest since 2010 while the DHS workforce responsible for providing the services is the smallest."

Additionally, the federal report documents contract provider turnover as "high, particularly among contract providers, especially in urban areas of the state, leading to inconsistency for children

and families that negatively affects their access to services." (Final Report, p.5)

On co-placement (commingling) in group care: The Children's Bureau flagged concern about co-placement of child welfare children and youth committed to the juvenile justice system in the same group care settings, including children with developmental disabilities who have experienced trauma. The Bureau "encourages Iowa to fully assess the scope of this issue across the state." This concern did not affect CFSR conformity decisions but was documented as a significant practice concern (Final Report, p.5).

6.4.16 National Data Indicators — Contextual Figures (Round 3)

Provided as contextual information only; not used in conformity decisions (Final Report, App. A):

Recurrence of maltreatment: Iowa RSP 14.1% (national performance: 9.5% or lower).

Maltreatment in foster care: Iowa RSP 19.77 per 100,000 days (national: 9.67 or lower).

Re-entry to foster care in 12 months: Iowa RSP 9.5% (national: 8.1% or lower).

Placement stability (moves per 1,000 days): Iowa RSP 3.15 (national: 4.44 or lower) — Iowa performed BETTER than national performance on placement stability.

Permanency in 12 months (new entrants): Iowa RSP 42.5% (national: 42.7%).

Permanency for children 12–23 months in care: Iowa RSP 69.2% (national: 45.9%) — Iowa above national.

Permanency for children 24+ months in care: Iowa RSP 46.8% (national: 31.8%) — Iowa above national.

(Final Report, Appendix A, pp.A-7 to A-8)

NOTE: Iowa's placement stability and long-term permanency indicators are above national performance in Round 3, while maltreatment recurrence and maltreatment in foster care remain significantly above national performance. These data are contextual and are not incorporated into conformity determinations under Round 3 methodology.

6.4.17 Round 3 Performance Improvement Plan

The Round 3 PIP was submitted multiple times: May 1, 2019; October 31, 2019; February 14, 2020; and resubmitted May 15, 2020. As of the submission date of May 13/14, 2020, the fields for "Date Approved," "PIP Effective Date," "End of PIP Implementation Period," and "End of Non-Overlapping Year" were left blank — meaning the PIP was submitted but had not received federal approval at the time of this document (PIP cover page, p.2).

6.4.17.1 Workforce Data — State Self-Documentation at Time of PIP

The PIP contains the most specific workforce data available in the three-round record:

Iowa had 342 Social Work Case Manager staff in SFY 2016. At time of PIP submission in 2019, the count was 296 YTD — a 13% decline. During the same period, the average caseload per worker increased from 30.6 to 36.5 — a 19% increase. (PIP, Section 1, p.4)

A staffing growth table documenting the period between the SFY 2018 onsite review and the May 2020 resubmission shows:

- Case Managers (SW2): 312 (SFY18) → 336 (SFY20), an 8% increase (24 staff)
 - Abuse Investigators (SW3): 205 (SFY18) → 248 (SFY20), a 20% increase (43 staff)
- (PIP, Strategy 1.2, p.30)

This growth occurred after the onsite review period and is documented as part of a four-year plan to increase front line staffing levels as budgets allow. The PIP attributes timeliness failures in part to workload issues: workers "prioritizing their workload, using the report information and local knowledge of a family to determine urgency to make contact." (PIP, p.30)

6.4.17.2 PIP Four Goals and Core Strategies

The Round 3 PIP organized reform around four goals (PIP, Table of Contents, pp.3 and 16–21):

Goal 1: Keep Children Safe at Home with Their Families Whenever Possible

Targeted outcomes: Safety 1 and Safety 2

- Strategy 1.1: Ensure child safety at each case stage and improve safety and risk assessment
- Strategy 1.2: Increase timely face-to-face contact with child victims; supervisor-CPW consultation when delayed
- Strategy 1.3: Implement the Safe 4 Home Initiative (4 questions) statewide

Goal 2: Improve Time to Permanency and Time to Safe Reunification

Targeted outcomes: Permanency 1, Permanency 2; Statewide Information System

- Strategy 2.1: Develop resources and training to identify, locate, and engage all fathers
- Strategy 2.2: Increase timely permanency through improved quality legal representation (NACC Redbook training using IV-E funds)
- Strategy 2.3: DHS workers enter placement data within 3 business days of placement/placement change

Goal 3: Children Experience Optimal Well-Being Through Improved Parental Capacity

Targeted outcomes: Well-Being 1, 2, 3; Service Array; Staff and Provider Training; Case Review System

- Strategy 3.1: Early family engagement in assessment and needs identification
- Strategy 3.2: Effectively engage with substance-using parents (Family Drug Courts + Infusion Project expansion)
- Strategy 3.3: Develop knowledgeable and supportive supervisors to reduce turnover and sustain practice change

Goal 4: Improve and Increase Collaborative Joint CQI System and Interventions

Targeted outcome: Quality Assurance Systemic Factor

— Strategy 4.1: Implement a joint CQI process between DHS and the Court Improvement Project (CIP) to create an integrated statewide feedback loop

6.4.17.3 Root Cause Analysis — Stakeholder-Documented

A root cause brainstorming group of approximately 100 stakeholders convened February 6, 2019 following the CFSR Final Report. The following categories of root causes were documented in the stakeholders' own language (PIP, Section 3, pp.13–14):

Workforce-related: "High caseloads, resulting in lack of time for quality visits, concurrent planning, training of staff." "High turn-over (DHS and Providers) relates to adequate service provision to family."

Lack of access or gaps to needed services: "Lack of services for mental health and substance abuse." "Lack of foster/adopt homes, older children/teen especially."

Lack of collaboration across agencies: "Working in silos, lack of communication with providers." "Lack of partnering between DHS and Court regarding family recommendations/decisions."

Lack of understanding of safety versus risk: "Lack of common understanding/definition across the system of the difference between safety and risk." "Inconsistent application of safety vs risk concerns related to removal of children and when to reunify."

Policy/procedure concerns: "THV-length of 6 months affects data and definition." "New founded reports based on past or substance use when child may be there (dangerous)."

6.4.17.4 Key Evidentiary Finding — Safety Plan Misuse Admission

The PIP documents an internal analysis revealing that safety plans were routinely misused:

"Safety plans used as 'promissory notes' with parents agreeing to abstain from identified behavior." Safety plans "were not consistently used to address safety issues, but to clarify expectations of behavior or actions a caretaker is to demonstrate." (PIP, Strategy 1.1, p.22–23)

Additionally: "Safety plans were often used to establish 'next steps' for families, rather than to address issues of imminent danger." (PIP, p.23)

The federal reviewers and Iowa's own internal case review both confirmed: in 29 of 32 (91%) applicable cases for Item 3B (ongoing assessments that accurately assessed all risk and safety concerns), assessments were "not sufficiently in-depth." (PIP, p.22)

This state self-admission establishes that the formal safety planning mechanism — the primary tool for managing child safety without removal — was functionally inoperative as an actual

safety instrument throughout the period of federal oversight.

6.4.17.5 Substance Abuse — Infusion Project and Family Treatment Court Data

The PIP documents Family Treatment Court (FTC) outcomes from the 13-year Iowa program as of March 2019, covering 1,385 families and 2,630 children:

- Children remaining safely in homes at case closure when removal was at risk: 74%
- Reunification rate in FTC removal cases: 73%
- Average length of stay in care for FTC children: 334 days (11.1 months), below the state average of 13.2 months for substance-involved cases
- No recurrence of maltreatment within 6 months: 96%
- Parents accessing substance abuse treatment: 95%

(PIP, Strategy 3.2, pp.49–50)

These outcomes are documented as evidence for expanding the Infusion Project model — a less intensive alternative for moderate-risk families not requiring full FTC intensity. The PIP planned addition of 2 new Infusion Project sites, with implementation target of February 2021.

(PIP, Key Activities, p.51)

6.4.17.6 Father Engagement — Quantified Disparity

The PIP documents a specific, quantified mother-father engagement disparity from Iowa's own case review data collected April–September 2018 (PIP, Strategy 2.1, p.38):

Item 12 (Comprehensive Assessment): Mother — 74%; Father — 61%

Item 12 (Provision of Services): Mother — 58%; Father — 40%

Item 13 (Active Involvement in Case Planning): Mother — 63.8%; Father — 50%

Item 15 (Worker Visits — Frequency): Mother — 89%; Father — 68.8%

Item 15 (Worker Visits — Quality): Mother — 85.7%; Father — 73.3%

Item 8 (Parent Visits with Child — Frequency): Mother — 89.3%; Father — 68.8%

Item 8 (Parent Visits with Child — Quality): Mother — 85.7%; Father — 73.3%

The PIP attributes this gap to "culturally ingrained perception of the mother as the primary caregiver" and documents that "fathers as a group are significantly marginalized as key participants in the child welfare system regardless of whether a non-resident father or part of a two-parent family." (PIP, p.38)

6.4.17.7 Item-Level Performance Data From PIP (CFSR Case Review Table)

The PIP contains the complete item-level performance table from the CFSR onsite review (PIP, Section 3, p.11). The following are the Strength ratings for key items not previously

documented in this section:

Item 3 (Risk and Safety Assessment and Management): 50.77% Strength / 49.23% ANI

(65 applicable cases; this is the lowest-performing safety item in the round)

Item 12 (Needs and Services of Child, Parents, Foster Parents): 44.62% Strength / 55.38% ANI

— Item 12A (Needs Assessment/Services to Children): 66.15% Strength

— Item 12B (Needs Assessment/Services to Parents): 44.07% Strength

— Item 12C (Needs Assessment/Services to Foster Parents): 84.85% Strength

Item 13 (Child and Family Involvement in Case Planning): 49.21% Strength / 50.79% ANI

Item 14 (Caseworker Visits with Child): 50.77% Strength / 49.23% ANI

Item 15 (Caseworker Visits with Parents): 25.42% Strength / 74.58% ANI

Item 17 (Physical Health): 59.09% Strength

Item 18 (Mental/Behavioral Health): 55.81% Strength

Outcome-level performance:

Safety Outcome 2: 50.77% Substantially Achieved (SA)

Well-Being Outcome 1: 38.46% SA

Well-Being Outcome 3: 47.54% SA

(PIP, Section 3, p.11)

AUDIT RELEVANCE: Item 15 (Caseworker visits with parents) at 25.42% Strength is the lowest single-item performance score in the entire Round 3 case review — lower even than the 10% documented in Round 1. Item 12B (Needs Assessment/Services to Parents) at 44.07% confirms that the most critical service-delivery indicator for parental reunification was failing in over half of applicable cases. Well-Being Outcome 1 at 38.46% is the second-lowest score in the three-round record (only Round 1's 24% was lower).

6.4.17.8 Safety Plans — Third Category of Misuse

In addition to safety plans used as "promissory notes" (6.4.17.4), the PIP documents a third distinct category of misuse:

"Safety plans misnamed: At times there appeared to be a safety plan in place, but upon review of the content it was not a plan for maintaining ongoing safety for the child but rather a crisis plan for handling a hypothetical future situation to mitigate risk." (PIP, Strategy 1.1, p.23)

This three-category taxonomy — promissory notes, monitoring-only plans, and misnamed plans — documents that Iowa's safety planning mechanism had three structurally distinct failure modes operating simultaneously. The PIP's corrective response was to contract with the National Council on Crime and Delinquency (NCCD) to develop a new validated Structured Decision

Making (SDM) tool for Iowa. Contract timeline: NCCD engagement April 2020; SDM tool development and validation May 2020–November 2020; training April–May 2021.

(PIP, Strategy 1.1 Key Activities, pp.27–28)

AUDIT RELEVANCE: Iowa did not have a validated safety assessment tool at the time of the Round 3 onsite review in 2018. The contract to develop one was not executed until April 2020 — two years after the federal review documented the failure. The CIA (2023) independently recommended: "Implement a Structured Decision Making intake model" — confirming the SDM tool still had not been statewide-implemented five years after the NCCD contract was initiated.

6.4.17.9 Accountability Provisions

The Round 3 PIP contained no defined consequences for missing benchmarks. Accountability was framed exclusively as monitoring and data review. Strategy improvement goals were structured as theories of change, not measurable targets with enforcement mechanisms.

The PIP was resubmitted four times over approximately 13 months (May 2019 through May 2020) before reaching its final form — with the approval date still blank at resubmission. This submission history represents the most protracted PIP development timeline in the three-round record.

The PIP concludes: "Iowa's child welfare system is dedicated to practice improvements outlined in this plan, which will positively impact the children and families served by the system in Iowa."

No specific performance targets, improvement thresholds, or numeric goals are stated in the conclusion. (PIP, Conclusion, p.57)

6.4.18 Round 3 Safe 4 Home Pilot Data

The PIP documents results from the Safe 4 Home Initiative pilot (7 judges, December 2019 through March 2020):

- Total removal requests: 83; approved: 44 (53%)

- Of removals granted: relative placement 55%, suitable other 11%, foster 34%, shelter 0%

Prior period comparison (same judges, August–November 2019):

- Total removal requests: 99; all granted

- Relative placement 42%, suitable other 9%, foster 34%, shelter 13%

(PIP, Strategy 1.3, p.35)

This data point demonstrates that a structured danger-identification tool applied to removal decisions — asking four questions before removing a child — resulted in 47% of requested removals being denied and an increase in relative placement from 42% to 55%. This is documented evidence that the state's prior removal practice did not consistently apply such a

standard.

6.5 LONGITUDINAL SUMMARY — KEY INDICATORS ACROSS ROUNDS 1–3

(All figures from federal Final Reports unless noted)

Outcomes failed:

Round 1 — 5 of 7 | Round 2 — 7 of 7 | Round 3 — 7 of 7

Systemic factors NOT in substantial conformity:

Round 1 — 4 of 7 | Round 2 — 2 of 7 | Round 3 — 5 of 7

NOTE: The decline from Round 2 (2 systemic factors failing) to Round 3 (5 failing) represents a documented systemic regression despite an intervening PIP. The Case Review System and Staff and Provider Training systemic factors had zero Strengths in Round 3.

Safety Outcome 2 (children safely maintained in homes):

Round 1 — PASSED (93.5%) | Round 2 — FAILED (63.1%) | Round 3 — FAILED (51%)

Trajectory: One of the two outcomes Iowa passed in Round 1; declined to 63.1% in Round 2 and 51% in Round 3. Successive all-outcome failure with this outcome declining each round.

Caseworker visits — children (Item 14/19):

Round 1 — 10% Strength | Round 2 — 65% Strength | Round 3 — 51% Strength

NOTE: Round 3 (51%) represents a decline from Round 2 (65%), indicating the Round 2 improvement did not hold.

State self-report gap in Round 2: 88% vs. federal finding of 65%.

Caseworker visits — parents (Item 15/20):

Round 1 — 23% Strength | Round 2 — 43% Strength | Round 3 — 25% Strength

NOTE: Round 3 (25%) is essentially equivalent to Round 1 (23%), erasing all Round 2 gains.

Well-Being Outcome 1 (families have enhanced capacity):

Round 1 — 24.0% | Round 2 — 40.0% | Round 3 — 38%

Round 3 represents a decline from Round 2.

Permanency Outcome 1 (permanency and stability):

Round 1 — 50.0% | Round 2 — 37.5% | Round 3 — 45%

All three rounds below 50%.

Well-Being Outcome 3 (physical and mental health):

Round 1 — 78.7% | Round 2 — 88.1% | Round 3 — 48%

Round 3 (48%) represents a dramatic decline from the trajectory.

Risk and Safety Assessment (Item 3):

Round 2 — 65% Strength (from ANI to measurement) | Round 3 — 51% Strength

Iowa's own internal case reviews for SFY 2017 showed 62% — close alignment but below target.

Caseworker visits — fathers specifically (sub-item data):

Round 3 only: Frequency with fathers — 26%; Quality with fathers — 28%

Fathers involved in case planning — 50% of applicable cases

Fathers' relationships promoted — 50% of applicable cases

The father engagement gap is documented as a persistent systemic failure across all three federal reviews.

Substance abuse as primary case-opening reason:

Round 1 — 48% of 50 federal cases | Round 2 — 42.5% of 40 foster care cases

Round 3 — Not directly reported as case-opening reason in federal Final Report.

Round 3 contextual data: Recurrence of maltreatment RSP 14.1% (vs. 9.5% national); maltreatment in foster care RSP 19.77 (vs. 9.67 national); re-entry RSP 9.5% (vs. 8.1% national).

QA discrepancy and systemic decline:

Round 1 — QA system did not exist statewide; rated ANI.

Round 2 — State reported 88% visit compliance; federal found 65% (23-point gap); state attributed to training and scoring oversight failures; rated ANI.

Round 3 — State self-rated QA as a "strength"; federal rated it ANI. CQI system has "no functional feedback loop." Data reports underutilized or inaccessible to frontline staff.

PIP completion certified, outcomes deteriorated:

Round 1 PIP — all goals certified as met at end of implementation period.

Round 2 result — all 7 outcomes failed.

Round 2 PIP — all outcome improvement targets designated "to be determined."

Round 3 result — all 7 outcomes failed again; systemic factors regressed from 2 failing to 5 failing.

Workforce trajectory:

Round 1 — avg 55 cases per worker; 11:1 supervisory ratio.

Round 2 — improved to 30 per worker (2008) then reversed; "caseloads risen and expected to continue to rise" at time of review.

Round 3 — "Caseload sizes are the largest since 2010 while the DHS workforce responsible for providing the services is the smallest." High turnover among contract providers.

Drug testing concerns — first formal federal documentation:

Round 3 Final Report: Stakeholders indicated "possible overuse of drug testing in Iowa, such as even when behavioral indicators of substance abuse are not present." Limited drug testing locations and hours documented as a statewide service access barrier.

Statewide Information System — regression:

Round 2 — in substantial conformity.

Round 3 — NOT in substantial conformity. Iowa's system cannot "accurately and readily identify the location of children in foster care." This represents a systemic regression in data infrastructure between rounds.

Case Review System — catastrophic failure:

Round 2 — in substantial conformity.

Round 3 — NOT in substantial conformity; ALL 5 items rated ANI. This is a full systemic collapse of a factor Iowa had been passing. Permanency hearings combined with TPR hearings, periodic reviews inconsistent, TPR filing untimely — all confirmed by federal review.

6.6 SOURCE VERIFICATION STATUS

Round 1 Statewide Assessment (pp.1–155): FULLY VERIFIED

Round 1 PIP (pp.1–145): FULLY VERIFIED

Round 1 Federal Final Report (October 14, 2003): FULLY VERIFIED

Round 2 Statewide Assessment (pp.1–198, March 2010): FULLY VERIFIED

Round 2 Federal Final Report (January/February 2011): FULLY VERIFIED

Round 2 PIP (submitted September 2, 2011): FULLY VERIFIED

Round 3 Statewide Assessment (pp.1–149, February 15, 2018): FULLY VERIFIED

Round 3 Federal Final Report (2018): FULLY VERIFIED

Round 3 PIP (submitted May 13/14, 2020; approval date blank): FULLY VERIFIED

6.7 CROSS-REFERENCES

Section 1.3 — Key Findings Overview (longitudinal synthesis)

Section 1.5 — Recurring Systemic Deficiency Patterns

Section 7 — CFSR Round 4 Structural Integrity Analysis

Section 9 — State Self-Admission Analysis

Attachment A2 — CFSR Round 1 source documents

Attachment A3 — CFSR Round 2 source documents

Attachment A4 — CFSR Round 3 source documents

Attachment A31 — Systemic Failure Matrix

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.